

Late Lichen Sclerosus Findings and Management

The loss of labia minora and midline agglutination of the clitoral hood are commonly observed in advanced lichen sclerosis.

Topical therapy with ultrapotent glucocorticoids, such as **clobetasol propionate ointment** applied once or twice daily, typically alleviates symptoms within a few days. However, several months of continuous treatment are necessary for complete resolution of clinical findings.[24,25] Most patients require 3 to 5 months of daily application to achieve maximal improvement. Monthly follow-up during active treatment is recommended to monitor for both therapeutic response and potential steroid-induced side effects, allowing for timely tapering of application frequency.

As adjunctive therapy during the first one to two weeks, sedating doses of an **antihistamine** or a **tricyclic antidepressant** can help control pruritus and promote healing.

Lichen sclerosis is generally not cured by treatment, and long-term maintenance is often needed. One study reported an 84% recurrence rate after discontinuation of therapy.[26]

Differential Diagnosis of Vulvar Lichen Sclerosus

Most Likely

- Lichen simplex chronicus
- Lichen planus
- Vitiligo

Consider

- Immunobullous disease
- Contact dermatitis

Always Rule Out

- Squamous cell carcinoma

Despite the necessity for prolonged therapy, there is no established optimal maintenance regimen. Many patients maintain good control with thrice-weekly application of an ultrapotent topical glucocorticoid indefinitely.

No other treatment matches the rapid and significant efficacy of topical ultrapotent glucocorticoids. Alternative medical therapies have mostly been evaluated in open-label studies. **Tacrolimus**[27–29] and **pimecrolimus**[30–31] have shown effectiveness in some cases (see Chap. 235). Topical **testosterone**, **progesterone**, and **estrogen** have demonstrated minimal benefit—comparable to vehicle or lubricant alone[32–34]—and may even increase irritation.[35] **Topical tretinoin 0.025%** and **calcipotriol** may improve symptoms in certain patients, but their use is limited by potential irritation.[36,37] **Oral retinoids** have been beneficial in select cases.[38] Traditional Chinese medicine, including herbal remedies and acupuncture, has been widely reported in Chinese literature to improve symptoms.[39]

Surgical interventions such as **cryotherapy**, **vulvectomy**, **carbon dioxide laser vaporization**, **pulsed dye laser therapy**, and **photodynamic therapy** have been proposed for lichen sclerosus treatment.[40–44]

Vulvar Fissures

Small linear fissures on the vulva are frequent causes of **itching**, **stinging**, and **dyspareunia**.^[45] Clinically, they may appear as painful erythematous lines within skinfolds or as distinct linear erosions.

The most recognized type occurs at the **posterior fourchette**, where splitting during intercourse leads to sharp tearing pain followed by burning and stinging upon exposure to alkaline semen. Although healing is rapid, recurrence with coitus is common. The etiology remains unclear. The condition often arises suddenly after years of pain-free intercourse. Preventive measures include adequate lubrication and positioning the woman in the superior (on-top) position to reduce trauma. If conservative measures fail, **surgical excision**

(perineoplasty) may be required—removing the vertical fissure and closing the defect with advancement of vaginal mucosa to cover the area.[46]

Recurrent fissuring within skinfolds, particularly in the **interlabial sulci**, is a common source of vulvar stinging and irritation. Women typically present with chronic discomfort exacerbated by friction or moisture.